



PAWSIC

POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

A CNA Q&A GUIDE

Skin in the *Game*

Real questions from Certified Nursing Assistants, answered by PAWSIC clinical experts. A quick-reference companion to the video series on skin health and wound care in post-acute settings.

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What's Inside

Eight questions CNAs and aides ask most, with the experts' answers.

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☆ ONE RULE ACROSS EVERY WOUND

All wounds must be kept covered. If you find a wound exposed to the air, report it. Old "let it breathe" thinking is out of date, and an uncovered wound risks infection and slower healing.

ASSESSMENT & WARNING SIGNS

What a Skin Check Should Look Like

Question 1: How should a skin check look for a CNA in a facility or a home health aide?

A good skin check uses all of your senses. The old teaching that "all skin is pink" misses problems on people with darker or lighter skin tones, so the first thing to look for is a color that is different from the rest of that person's own skin. A darker or lighter area compared to the surrounding skin can be an early concern.

CHECK ALL OF THE SKIN

- Problems are most common on the heels, ankles, hips, and the sacral or buttock area, but they can happen anywhere. Do a full-body check: move the hair to see the back of the head, open every skin fold, and spread apart rolls, including on the legs.
- Look under any device you are allowed to remove, such as a heel boot worn for sleep. Even devices meant to prevent wounds can sometimes cause them.

USE EVERY SENSE

- Feel for anything that feels different or odd, like a water balloon or a waterbed under the skin. That can mean a pressure injury is brewing, so report what feels wrong, not only what looks wrong.
- Notice if an area feels warm compared to the skin around it, or if the resident says "ow" during a bath. Both are worth reporting.

EARLY SIGNS TO REPORT

A color change in one area versus the rest, skin that starts to blister or separate, or skin that looks sheared. Any abnormality is worth reporting, and the earlier it is caught, the better the chance of healing.

Report with descriptive words: "redder than the skin around it," "squishy like a water balloon," "more fragile, folds more." That detail helps the nurse and therapist know what they are walking into.

TREATMENTS & PRODUCTS

Wound VACs, Explained

Question 2: What is a wound VAC, how does it work, and where should it sit?

The clinical name is negative pressure wound therapy. "Wound VAC" is just one brand name; devices range from large canisters to units small enough for a pocket, but the science is the same. Filler (foam or special gauze) goes into a deep, draining wound, a cover seals it, and a tube provides gentle suction that pulls drainage out and holds the filler against the wound. It can be placed anywhere on the body.

WHAT A CNA SHOULD WATCH FOR

- **Placement and tubing.** A drainage canister is generally kept lower than the wound, but safety comes first. The tubing may be routed or "bridged" away from areas the resident lies on, so where the tubing sits is not where the wound is.
- **The sound.** Negative pressure is an active treatment: it should be working. If it goes quiet, the foam rises above the wound, or an alarm sounds, the suction may have stopped or there may be a leak.
- **Showers and dressing changes.** You may get an order to bathe the resident with the tubing detached and the foam left in place. That is a normal part of a dressing change and makes removal more comfortable.
- **If it unplugs briefly** during care, do not panic; most units run on battery for hours. If it happens constantly, report it. When in doubt, ask the nurse.

REPORT RIGHT AWAY

- Frank red blood in the tube or canister (a little pink tint can be normal).
- An alarm, or a device that is no longer suctioning.
- A dressing seal that has broken. Seals can take a long time to get, so tell the nurse rather than hiding it.

Doing a bed bath over the dressing? Check with your nurse first, because keeping the seal intact is what makes the therapy work.

PREVENTION & POSITIONING

Preventing Moisture and Fungal Skin Damage

Question 3: How do we keep skin folds and "nooks and crannies" healthy?

Every skin fold and contracture needs the same three steps: clean it, inspect it, and carefully pat it dry. Dab to dry, do not scrub, and do not stuff material into a fold, because that can create a pressure injury.

USE THE RIGHT PRODUCTS, THE RIGHT WAY

- Use only products the facility stocks or the physician orders. They are pH balanced for fragile skin.
- Do not bring lotions from home. Fragrances and the wrong pH can cause the very skin problems you are trying to prevent.
- Some thin, pH-balanced, often antimicrobial products are made to go into skin folds. Use only what is ordered, and ask how much and where.
- Do not keep layering powder or cream on top of the old layer. Remove what is there, dry the skin, then reapply.

YOU ARE THE EYES ON THE SKIN

If you see moisture building up in a fold, or a fungal look or odor, report it. The nursing team may not see it as often as you do, and there are things the team can do about it.

Dab to dry is still the rule: remove everything gently, dry as best you can, then apply only what is ordered.

TREATMENTS & PRODUCTS

Why Clinicians Choose Certain Products

Question 4: Why does one resident get a certain cream and another gets something different?

There are thousands of skin products across a few categories, and different ingredients suit different skin. Sensitive skin may need something thinner, dry skin may need a cream with less water and alcohol, some products carry an antimicrobial or antifungal, and some are heavier pastes for skin that is constantly moist. Each product is selected for a reason.

WHAT THIS MEANS FOR YOU

- Follow the care plan. Do not substitute, and do not add scented products from home for a vulnerable population with fragile skin.
- If you are not sure how much to use, where to apply it, or how to apply it, ask before you assume. Those answers matter.

⊕ WHERE TO FIND ANSWERS

- **Skilled nursing:** start with the nurse you report to or the unit manager. If it is a facility-wide question, ask the educator, who can bring in a vendor to teach the team about the products on your shelves.
- **Home health:** check the care plan built by the RN case manager. If it is not covered there, follow your agency's chain of command.

The key is simple: stop and ask when you are unsure of a product or how to use it. It is always better to ask than to assume.

PREVENTION & POSITIONING

Repositioning Beyond the 2-Hour Rule

Question 5: Are there better ways to reposition people, and how do I cushion bony residents?

The old idea that everyone turns every two hours is not based on research. Repositioning is individual, almost like a dose for that specific person. If a resident already has a pressure injury, assume every one to two hours. If not, find out from the nurse or care plan how often that person should be turned.

WORK IT INTO CARE YOU ALREADY GIVE

- Brief changes, toileting, and help with meals and daily activities all count as turning and repositioning. Do it as you go, and document it.
- Ask the nurse or therapist about positioning aids and a physical therapy consult. The right device lowers injury risk for you and the resident.
- If a positioning approach is not working, for example a resident keeps kicking off a boot, tell the team so a different option can be found.



PROTECT THE HEELS

Anyone who cannot lift their own legs should never have their heels resting directly on the bed, or on the floor in a recliner. Use a pillow under the calf or an ordered heel-offloading boot. Heel pressure injuries can develop in two to three hours or less. And even on a specialty or low-air-loss mattress, the resident still must be turned.

An offloading device is any aid, such as a wedge or boot, that removes or reduces pressure from a body part. Not sure how to apply one? Ask your nurse, or the therapist for a specialty device.

ASSESSMENT & WARNING SIGNS

Do Wounds Have a Smell?

Question 6: Do wounds smell, and what should I do about it?

A wound that is covered and clean should not have a strong odor. Some odor can be normal: a dressing doing its job, or a negative pressure device, can carry a mild smell, and certain palliative or cancer wounds are hard to control. What you are listening for is a change, or a strong, foul, sweet, or fecal smell that is not that person's baseline.

WHAT TO DO

- First, check that the dressing is still in place and intact. A loose dressing is often the clue.
- Do not smell a dressing on purpose. Just notice a change and report it.
- Describe the smell so the nurse gets a picture: strong, foul, sweet, fecal, or "hits you as soon as you enter the room," and when you noticed it.
- Take socks off every day. It is a common place to find hidden wounds and fungal infections.

ODOR IS A QUALITY-OF-LIFE ISSUE

If a wound odor is bothering a resident or their family, report it. With thousands of dressing options, there may be a better choice for odor control. A change in smell can also be the first sign of a bigger problem, so speak up.

ASSESSMENT & WARNING SIGNS

What the Braden Score Means

Question 7: I keep hearing about a Braden Score. What is it?

Do not get stuck on the number. What matters are the Braden categories, the risk factors for developing a pressure injury: moisture, nutrition, mobility, sensation and cognition, and friction and shear. The nurse assesses these on admission, often weekly for the first weeks, then quarterly and with any change of condition, and assigns an intervention for each risk factor.

HOW RISK FACTORS TURN INTO A CARE PLAN

- **Mobility:** a specialty bed or seating surface, heel offloading, or a program to get the resident moving.
- **Nutrition:** a dietitian evaluation and possibly supplements.
- **Moisture:** products for skin folds or for continence care.
- **Friction and shear:** techniques to reposition without adding injury.

YOUR VOICE MAKES THE PLAN WORK

Most interventions work for most people, but not everyone. You see the resident more than anyone, so you are the one who can tell the team when a device or schedule is not working for that individual. A device that offloads a high-risk area can add pressure somewhere else, so keep watching all of the skin.

ASSESSMENT & WARNING SIGNS

Wound, Pressure Injury, Skin Failure, Skin Tear

Question 8: What is a wound, and how do these types differ?

Skin is the largest organ of the body, and it is alive. A **wound** is skin that has been injured. That can happen from the outside, when something tears the skin, or from the inside, when the skin is not getting enough blood and begins to die. Sometimes the injury starts as discoloration before you can see an open area.

- **Pressure injury** (you may also hear older terms such as pressure ulcer or decubitus). Tissue dies when steady pressure starves the small blood vessels of oxygen, usually over bony areas. Devices such as catheters or oxygen tubing can cause a device-related pressure injury too. Movement and offloading are the prevention.
- **Skin failure** is a medical diagnosis. The skin, as an organ, begins to fail, generally at the end of life or when other organs are failing. It is not caused by pressure. Never assume it, and if a patient or family asks, refer them to the nurse or physician.
- **Skin tear** is usually a traumatic event, a bump or a brush, and it is the most common wound in older adults, whose skin is more fragile. Keeping skin clean and moisturized helps prevent it.

✓ PREVENTING SKIN TEARS

Moisturize everywhere except between the toes, keep skin from getting dry, handle limbs gently, consider long sleeves, and pad hard surfaces like wheelchairs and bed rails. A resident who has had one skin tear is at high risk for another, so be extra careful with lifting and handling.

These are all medical diagnoses that the clinical team assigns. Your job is to describe accurately what you see and report it. That is what lets the team act.

You Are the Eyes and Ears

Across every question, the answer came back to the same thing: the CNA and the aide see the resident more than anyone else on the team. What you notice, and what you report, is what lets nurses and therapists act in time. Your voice matters more than you know.

When you are unsure, be brave and ask. Talk to your nurse, talk to the therapist. There is no wrong time to check.

★ REMEMBER ONE THING

All wounds, in every setting, must be kept covered. If you walk in and a wound is open to the air, something is not right. Report it.

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